Trastuzumab IV · every 3 weeks

NCCP 00200 · Source reconciled; independent clinical, pharmacy and patient review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop.

STEP 1

Before your visit

Confirm the dose, symptoms, required blood results and echo due window.

STEP 2

Access and administration

A cannula, PICC or implanted port may be used according to the treatment plan. A port insertion is a separate procedure.

STEP 3

Treatment and observation

First infusion usually 90 minutes; later infusions may take 30 minutes if tolerated. NCCP observation is 6 hours from the start of the first infusion and 2 hours from the start of later infusions; ask about the unit’s approved policy.

STEP 4

After the visit

Tell the unit how late the dose is. A delay over one week can require re-loading after clinician review; do not assume every continuation visit needs a loading dose. This medicine does not routinely require a take-home pump.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit length

First infusion usually 90 minutes; later infusions may take 30 minutes if tolerated. NCCP observation is 6 hours from the start of the first infusion and 2 hours from the start of later infusions; ask about the unit’s approved policy.

Missed or delayed visit

Tell the unit how late the dose is. A delay over one week can require re-loading after clinician review; do not assume every continuation visit needs a loading dose.

Sources: NCCP 00200 v8 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient review pending.